

CHECKLIST

Phase 1: Exploration Checklist

Identifying Human Factors and Environmental Context

BOTTOM LINE

Assess internal needs, select an intervention, assemble the team, gauge staff and patient attitudes, confirm readiness (CARI $\geq$ 80), and secure funding. Skipping exploration is the most common reason implementations stall.

1. Internal Needs Assessment

- ☐ Review hospital guidelines for current PONV prevention and management.
- ☐ Verify PONV incidence is charted in Recovery, ward, and pre-discharge charts.
- ☐ Audit current PONV incidence rate (baseline: 30–50% of surgical patients).
- ☐ Track rescue antiemetic usage separately – not equivalent to PONV. (Gan, 2014)
- ☐ Confirm PONV risk is formally assessed pre-operatively.

Risk Factors Each factor adds 20% risk (Apfel, 1999): female · non-smoker · previous PONV/motion sickness · post-op opioids.

2. Intervention Selection

- ☐ Review all six modalities (see Intervention Comparison Card).
- ☐ Assess fit with local resources, staff skills, and patient population.
- ☐ First-time / nurse-led: default to **acupressure wristbands** (\$5, strongest evidence).
- ☐ Credentialed acupuncturist available? Consider body or auricular acupuncture.

Pooled PC6 Evidence Nausea: 47%→ 31% (RR 0.68) **HIGH** Vomiting: 33%→ 19% (RR 0.60) **HIGH** Rescue: 33%→ 20% (RR 0.64) **MODERATE** Lee 2015; Cheong 2013.

3. Team Composition

- ☐ Identify **Opinion Leaders** – those with formal/informal influence on attitudes.
- ☐ Recruit **Champions** – who will drive implementation past resistance.
- ☐ Engage **External Change Agents** – external experts who facilitate decisions.

- ☐ Ensure early adopters share professional/cultural backgrounds with target staff.

4. Staff Assessment

- ☐ Survey anaesthetists, nurses, midwives, surgeons, and administrative staff.
- ☐ Assess baseline knowledge of acupuncture/acupressure for PONV.
- ☐ Gauge willingness to use and to pursue further education.
- ☐ Identify perceived barriers (evidence, credentialing, equipment, time).

Benchmarks **USA:** 54% willing; 74% would pursue education; barriers: evidence 79%, providers 71%, equipment 49%. (Faircloth, 2014)
Aus: 42% believed effective; 81% would encourage post-education; 88% wanted training.

5. Organisational Readiness (CARI)

- ☐ Complete the Modified CARI assessment. (Barwick, 2011)
- ☐ Score 5 categories: system readiness, leadership, staff capability, plan maturity, training.
- ☐ Confirm total $\geq 80/100$ (min 15 per category). Address weakest area first if below.

6. Cost and Funding

- ☐ Estimate device costs (wristbands \$5; needles \$0.05; e-stim \$40–200; EA \$200–800).
- ☐ Calculate staff time for delivery/monitoring.
- ☐ Document current antiemetic costs. (Myle, 2016)
- ☐ Build cost-effectiveness comparison.
- ☐ Identify funding source.

7. Patient Perspectives

- ☐ Survey patient willingness to use acupuncture/acupressure.
- ☐ Provide education – willingness: 65% → 87%. (Weeks, 2017)
- ☐ Only 15% know acupuncture treats nausea at baseline.
- ☐ Incorporate patient preferences per guidelines.

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